

COXA 500 TABLET

Name and Strength of Active substance

Coxa 500 Tablet

Each tablet contains paracetamol 500mg.

Product Description

Coxa 500 tablet is a white round, uniform and scored on one side. The score line is not intended to divide the tablet into equal doses.

Pharmacodynamic

Paracetamol has analgesic and antipyretic effects. It may produce analgesia by inhibiting prostaglandin synthesis in the central nervous system and through a peripheral action by blocking pain impulse generation. The peripheral action may also be due to inhibition of prostaglandin synthesis or to inhibition of the synthesis or actions of other substances that sensitise pain receptors to mechanical or chemical stimulation.

Paracetamol probably produces antipyresis by acting centrally on the hypothalamic heat regulating centre to produce peripheral vasodilation resulting in increased blood flow through skin, sweating and heat loss. The central action probably involves the inhibition of prostaglandin synthesis in the hypothalamus.

Pharmacokinetic

Absorption

The oral absorption of paracetamol is rapid and almost completely absorbed from the gastro-intestinal tract with peak plasma concentration occurring about 0.5 to 2 hours after ingestion and may be decreased if taken after a high carbohydrate meal.

Distribution

Paracetamol is distributed rapidly throughout all tissues. Concentrations are comparable in blood, saliva and plasma. The volume of distribution of Paracetamol is approximately 1 L/kg bodyweight. At therapeutic doses protein binding is negligible.

Metabolism

Protein binding is not significant with doses producing plasma concentration below 60mcg/ml but may reach moderate level with high or toxic doses. Approximately 90% - 95% of a dose is metabolized in the liver, primarily by conjugation with glucuronic acid, sulphuric acid and cysteine. An intermediate metabolite, which may accumulate in overdosage after the primary metabolic pathways become saturated, is hepatotoxic and possibly nephrotoxic. The half-life is 1-4 hours and does not change with renal failure but may be prolonged in some forms of hepatic disease, the elderly and neonates and may be somewhat shortened in children. The duration of action is 3-4 hours.

Elimination

Elimination of Paracetamol is essentially through the urine. 90% of the ingested dose is eliminated via the kidneys within 24 hours, predominantly as the glucuronide (60 to 80%) and the sulphate (20 to 30%) conjugates. Less than 3% is eliminated in unchanged form.

Indication

For the relief of fever and reduce pain.

Route of Administration

For oral use only.

Recommended Dosage

Adults and children over 12 years: 1-2 tablets three to four times daily as required. Not more than 8 tablets in 24 hours unless instructed by a doctor.

Children under 12 years: Not recommended.

Contraindication

Hypersensitivity to the Paracetamol or to any of the excipients listed.

Warning and Precautions

Paracetamol should be given with care to patients with severe renal or severe hepatic impairment. The hazards of overdose are greater in those with non-cirrhotic alcoholic liver disease. Prolonged use except under medical supervision can be harmful. If symptoms more than 3 days, consult the doctor.

This preparation contains PARACETAMOL. Do not take any other paracetamol containing medicines at the same time.

Allergy alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of serious condition. If these reactions occur, stop use and seek medical assistance right away.

Interactions with other Medicaments

Cholestyramine: The speed of absorption of paracetamol is reduced by cholestyramine. Therefore, the cholestyramine should not be taken within one hour if maximal analgesia is required.

Metoclopramide and Domperidone: The absorption of paracetamol is increased by metoclopramide and domperidone. However, concurrent use need not be avoided.

Warfarin: The anticoagulant effect of warfarin and other coumarins may be enhanced by prolonged regular use of paracetamol with increased risk of bleeding; occasional doses have no significant effect.

Chloramphenicol: Increased plasma concentration of chloramphenicol.

Alcohol, hepatic enzyme inducers, hepatotoxic medications: May increase the risk of hepatotoxicity with single toxic dose or prolonged use of high doses of paracetamol

Non-steroidal anti-inflammatory drug (NSAIDs): Prolonged concurrent use of paracetamol may also increase the risk of adverse renal effects.

Pregnancy and Lactation

Pregnancy

A large amount of data on pregnant women indicate neither malformative, nor feto/neonatal toxicity. Epidemiological studies on neurodevelopment in children exposed to paracetamol in utero show inconclusive results. Epidemiological studies in human pregnancy have also shown no ill effects due to paracetamol used in the recommended dosage, but patients should follow the advice of their doctor regarding its use. If clinically needed, paracetamol can be used during pregnancy however it should be used at the lowest effective dose for the shortest possible time and at the lowest possible frequency.

Breastfeeding

Following oral administration, small amounts of paracetamol are excreted into breast milk, however not in a clinically significant amount. To date, there are no known undesirable effects or side effects during breast-feeding. Paracetamol can be administered during lactation at therapeutic doses.

Fertility

No detrimental effects on fertility upon normal use of Paracetamol are known

Adverse Effects/ Undesirable Effects

At therapeutic doses few undesirable effects occur.

Adverse events of paracetamol from historical clinical trial data are both infrequent and from small patient exposure. Accordingly, events reported from extensive post-marketing experience at therapeutic/labelled dose and considered attributable are tabulated below by system class. The following convention has been utilised for the classification of undesirable effects: Very common ($\geq 1/10$), common ($\geq 1/100$ to $< 1/10$), uncommon ($\geq 1/1,000$ to $< 1/100$), rare ($\geq 1/10,000$ to $< 1/1,000$), very rare ($< 1/10,000$), not known (cannot be estimated from available data). Due to limited clinical trial data, the frequency of these adverse events is not known (cannot be estimated from available data), but post-marketing experience indicates that adverse reactions to paracetamol are rare and serious reactions are very rare.

Post marketing data

Body System	Undesirable effect	Frequency
Blood and lymphatic system disorders	Thrombocytopenia Agranulocytosis	Very rare
Immune system disorders	Anaphylaxis Cutaneous hypersensitivity reactions including skin rashes and angioedema	Very rare
Skin and subcutaneous tissue disorders	Serious skin reactions	Very rare
Respiratory, thoracic and mediastinal disorders	Bronchospasm*	Very rare
Hepatobiliary disorders	Hepatic dysfunction	Very rare

* There have been cases of bronchospasm with paracetamol, but these are more likely in asthmatics sensitive to aspirin or other NSAIDs.

Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.

Overdose and treatment

Paracetamol can result in poisoning, particularly in elderly subjects, young children, patients with liver diseases, in cases of chronic alcoholism, in patients suffering from chronic malnutrition and patients using liver enzyme inducing agents. Overdose may be fatal in these cases.

Liver damage is possible in adults who have taken 10g or more of paracetamol. Ingestion of 5g or more of paracetamol may lead to liver damage if the patient has risk factors (see below).

Risk Factors

If the patient

- Is on long term treatment with carbamazepine, phenobarbitone, phenytoin, primidone, rifampicin, St John's wort or other drugs that induce liver enzymes. Or
- Regularly consumes ethanol in excess of recommended amounts. Or
- Is likely to be glutathione deplete e.g. eating disorders, cystic fibrosis, HIV infection, starvation, cachexia.

Symptoms

Symptoms of paracetamol overdosage in the first 24 hours are pallor, nausea, vomiting, anorexia and abdominal pain. Liver damage may become apparent 12 to 48 hours after ingestion. Abnormalities of glucose metabolism and metabolic acidosis may occur. In severe poisoning, hepatic failure may progress to encephalopathy, haemorrhage, hypoglycaemia, cerebral oedema, and death. Acute renal failure with acute tubular necrosis, strongly suggested by loin pain, haematuria and proteinuria, may develop even in the absence of severe liver damage. Cardiac arrhythmias and pancreatitis have been reported.

Treatment

Local treatment guidelines for Paracetamol overdose should be followed.

Immediate treatment is essential in the management of paracetamol overdose. Despite a lack of significant early symptoms, patients should be referred to hospital urgently for immediate medical attention. Symptoms may be limited to nausea or vomiting and may not reflect the severity of overdose or the risk of organ damage. Management should be in accordance with established treatment guidelines.

Treatment including emptying the stomach via induction of emesis or gastric lavage may be applied.

Treatment with activated charcoal should be considered if the overdose has been taken within 1 hour. Plasma paracetamol concentration should be measured at 4 hours or later after ingestion (earlier concentrations are unreliable).

Treatment with N-acetylcysteine may be used up to 24 hours after ingestion of paracetamol however, the maximum protective effect is obtained up to 8 hours post ingestion. Oral administration of NAC should not be combined with oral activated charcoal. If required the patient should be given intravenous-N-acetylcysteine, in line with the established dosage schedule. In general, intravenous administration of NAC is preferred and should be continued until paracetamol is no longer detectable. It is important to realize that intake of NAC up to 36 hours

after intake can improve prognosis. If vomiting is not a problem, oral methionine may be a suitable alternative for remote areas, outside hospital.

Liver tests have to be performed at the start of treatment and need to be repeated each 24 hours after treatment. In most cases, hepatic transaminases will return to normal levels within two weeks after intake of overdose with complete recovery of liver function. In rare cases, liver transplantation may be required.

Effects on Ability to drive and use machine

None.

Storage conditions:

Store below 30°C. Protect from light. Keep out of reach of children.

Dosage Forms and Packaging Available

1 X 10 tablet

10 X 10 tablet

50 X 10 tablet

100 X 10 tablet

Manufacturer

Medinfar Manufacturing, S.A.
Parque Industrial Armando Martins Tavares
Rua Outeiro da Armada, 5
Condeixa-a-Nova
3150-194 Sebal
Portugal

Product Registration Holder

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